THE MASTER MECHANIST'S COMPENDIUM OF EPONYMS

OPERATIONAL STATUS: CRITICAL SYSTEM REVIEW FOR NCLEX-RN CERTIFICATION

From the archives of the Blue Book & The Eponymous Wiki

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THE DIAGNOSTIC CODE
(MONITORING GAUGES)

STABLE/ EXPECTED

Operational. Document and Monitor. (e.g., Prssumptive Pregnancy Signs)

CRITICAL FAILURE

System Alert. Immediate Intervention Required. (e.g., Sepsis, Hemorrhage, Meningitis)

ACTION REQUIRED

Mechanist Intervention. Nursing prlority action needed immediately.

MECHANISM

Internal Logie. The "Why" behind the machine's failure.

CAUTION

Safety Risk c
Assessment

Based on Klimek Prioritization Protocols:
Is it Acute?
Is it Deadly?

ALERT: PRESSURE CRITICAL

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The Mainframe: Meningeal Irritation

Brudzinski's Sign

Flexion of the neck causes involuntary flexion of the hips and knees.

Kernig's Sign

Resistance/Pain when straightening the leg from a flexed hip position.

THE MECHANISM:
Inflammation of the meninges (Meningitis).

PRIORITY ACTION:
ISOLATION PRECAUTIONS IMMEDIATELY.

System Note: Insulation Failu NotebookLM

The Mainframe: Trauma & Reflexes

Battle's Sign

Ecchymosis over the mastoid process.

PRIORITY: BASAL SKULL FRACTURE.
Risk of CSF Leak.

Tech Details: Roboto Mono, DTFC[illegible], and susraitlles IIN EDkNSL 80E

Babinski Reflex

Stroke lateral sole -> Toes fan.

INFANT (<1 yr): NORMAL OPERATION.

ADULT: CNS LESION DETECTED. ABNORMAL.

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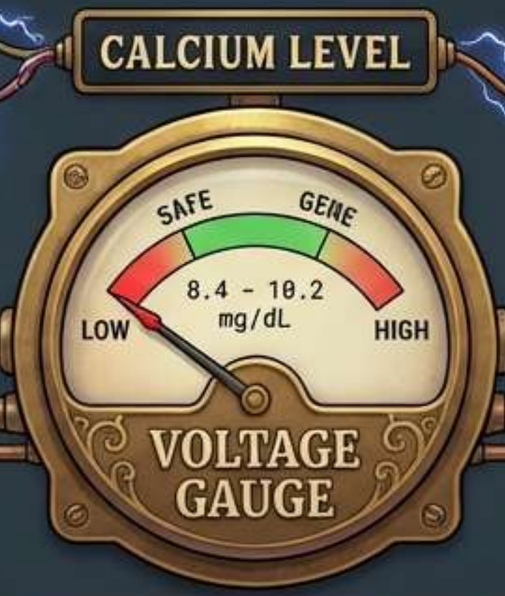
THE REGULATORS: HYPOCALCEMIA (LOW VOLTAGE)

CHVOSTEK'S SIGN

Facial nerve twitch upon tapping.



TROUSSEAU'S SIGN

Carpopedal spasm upon BP cuff inflation.

MECHANISM:
Tetany / Neuromuscular Excitability.

ACTION REQUIRED: SEIZURE PRECAUTIONS.
Airway Equipment at Bedside.

System Note: Low Voltage Detected. Neuromuscular M

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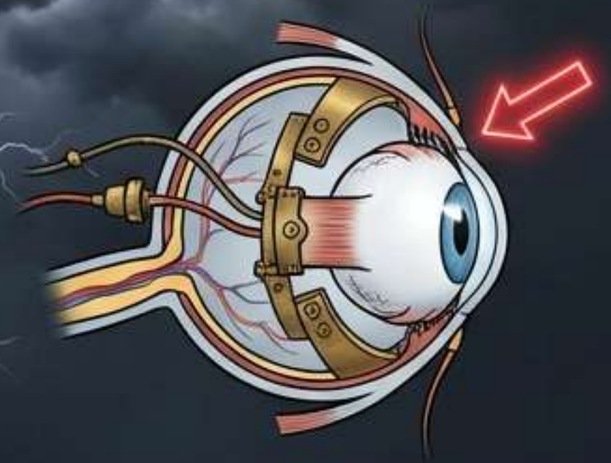
THE REGULATORS: THYROID & METABOLIC VENTING

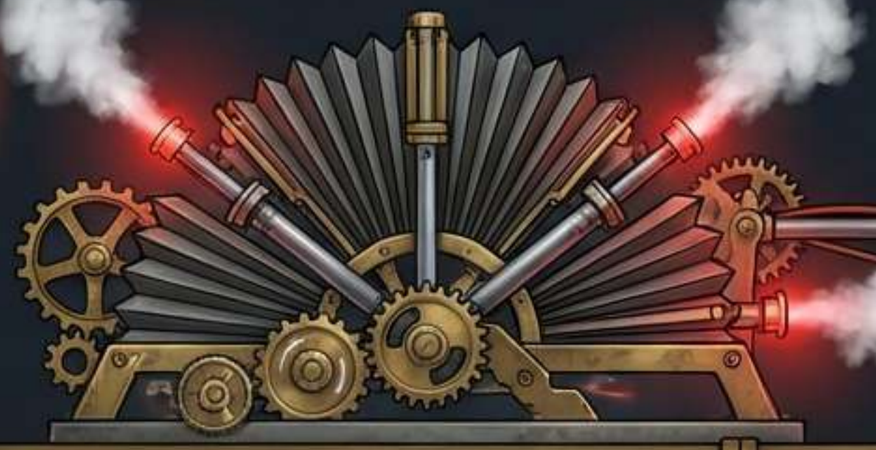


GRAVES' DISEASE
(HYPERTHYROID)

GRAEFE'S SIGN (Lid Lag):
Upper eyelid lags behind eyeball on downward gaze.

Monitor for Thyroid Storm.

METABOLIC ACIDOSIS
(THE SAFETY VALVE)



KUSSMAUL BREATHING

Deep, rapid respiration.
Blowing off CO_2 to reduce acid pressure.

LAB VALUES

pH: < 7.35 (ACIDIC)
HCO3: < 22 (LOW)

PRIORITY: DKA / SEVERE ACIDOSIS.

System Note: Acid Pressure Critical.
Venting in progress.

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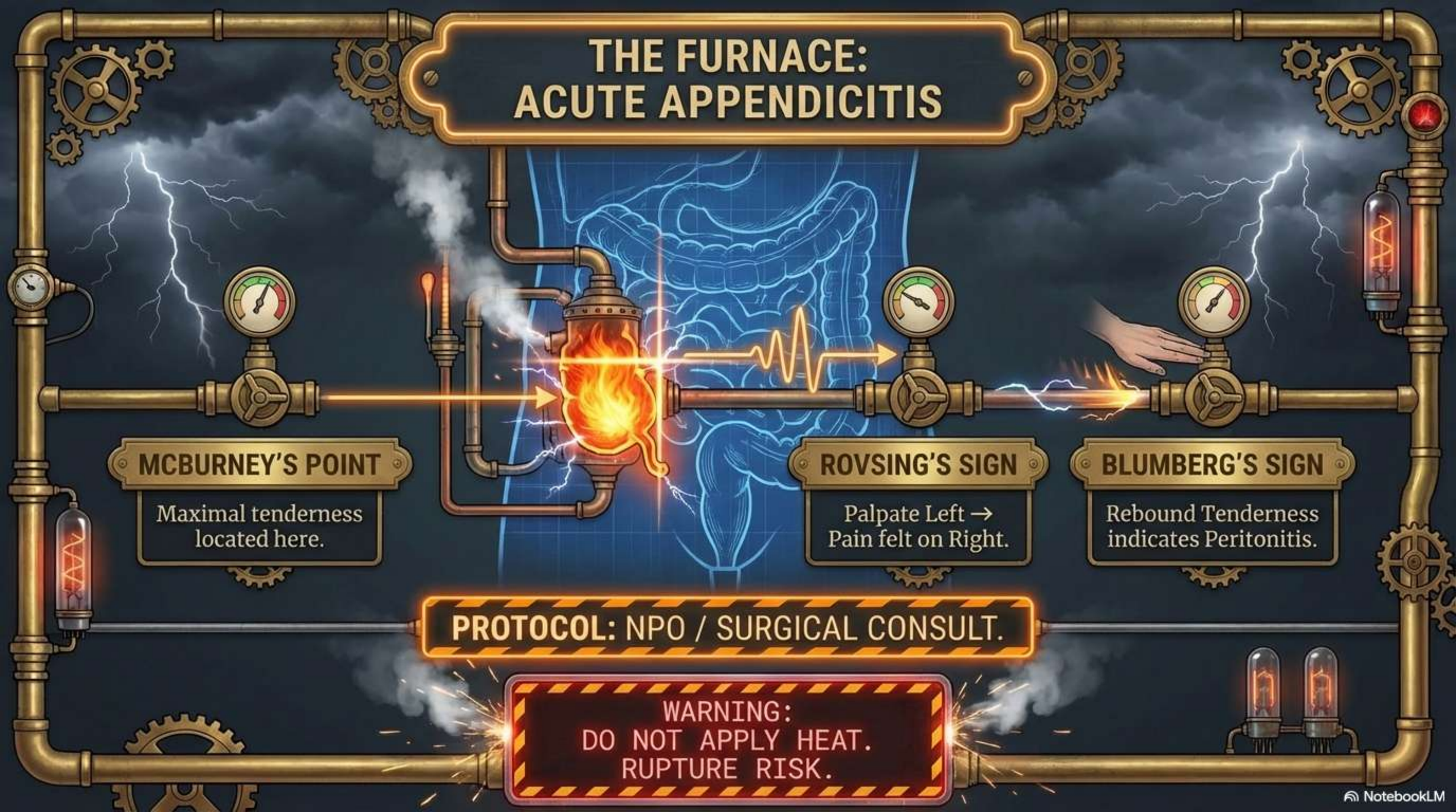
THE FURNACE: ACUTE APPENDICITIS



THE FURNACE: RUPTURES & LEAKS

CULLEN'S SIGN

Ecchymosis at Umbilicus
-> Pancreatitis or
Intra-abdominal bleed.

KEHR'S SIGN

Referred Pain
-> Ruptured Spleen or
Ectopic Pregnancy.

GREY TURNER'S SIGN

Flank Ecchymosis
-> Retroperitoneal Hemorrhage.

BLOOD PRESSURE

PRIORITY: CIRCULATORY SHOCK ASSESSMENT

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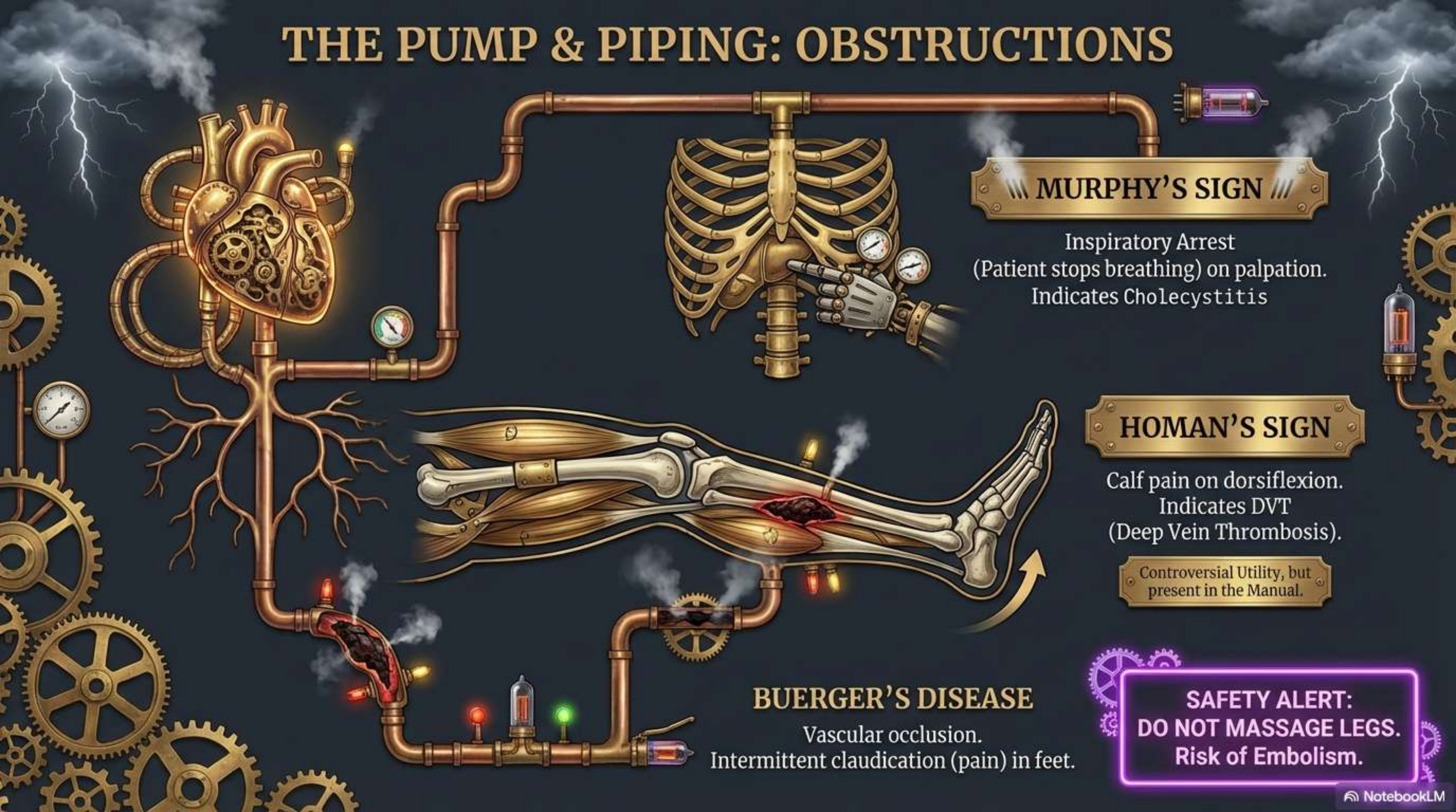
THE PUMP & PIPING: OBSTRUCTIONS



THE GENESIS CHAMBER: MATERNAL SIGNS

Normal Operational Indicators

1. **CHADWICK'S SIGN**
   Bluish-purple coloration of cervix. (Increased vascularity).
2. **GOODELL'S SIGN**
   Softening of the cervix.
3. **HEGAR'S SIGN**
   Softening of the lower uterine segment.

MECHANIST'S NOTE

STATUS: STABLE.

These are Presumptive/Probable signs of production.

Do not panic.
Do not intervene.

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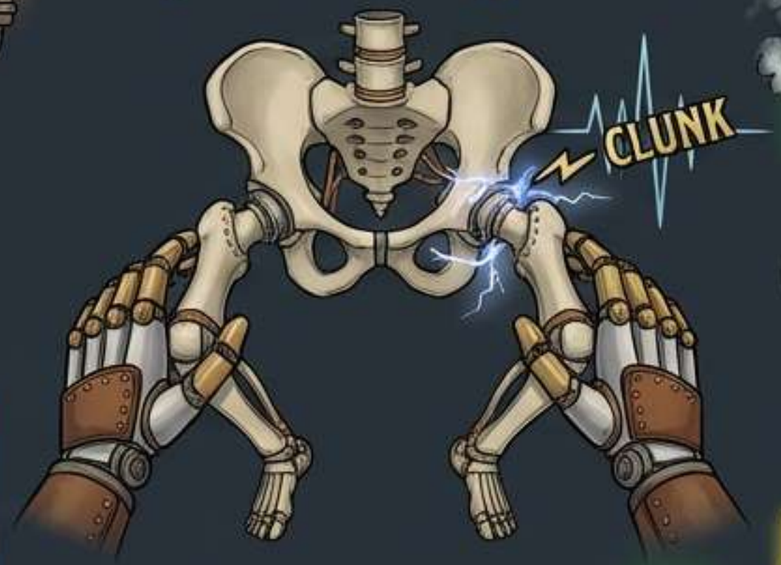
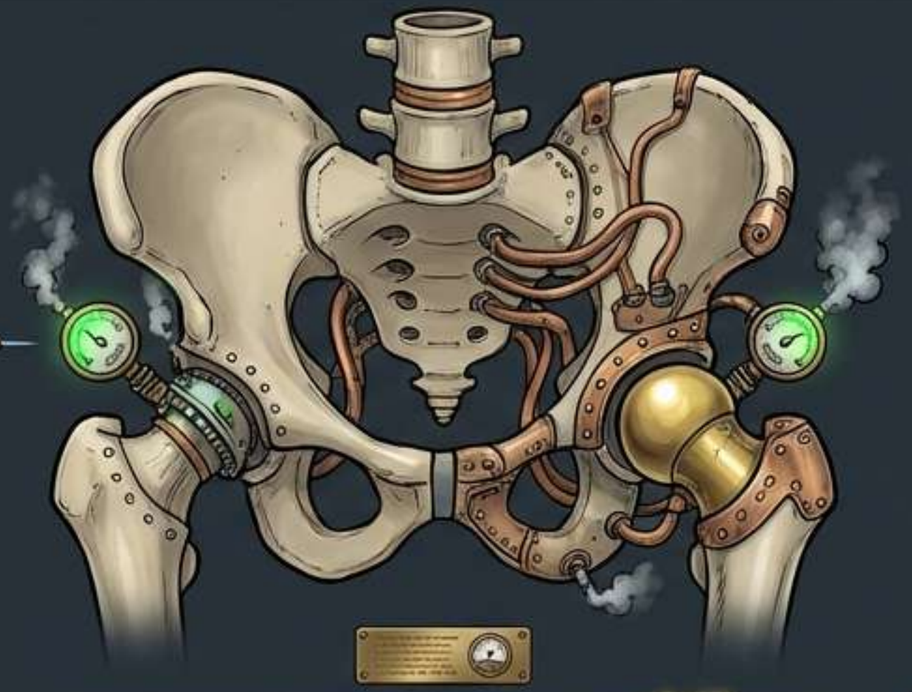
The Prototype: Structural Integrity (Hips)

"Ortolani's Click"

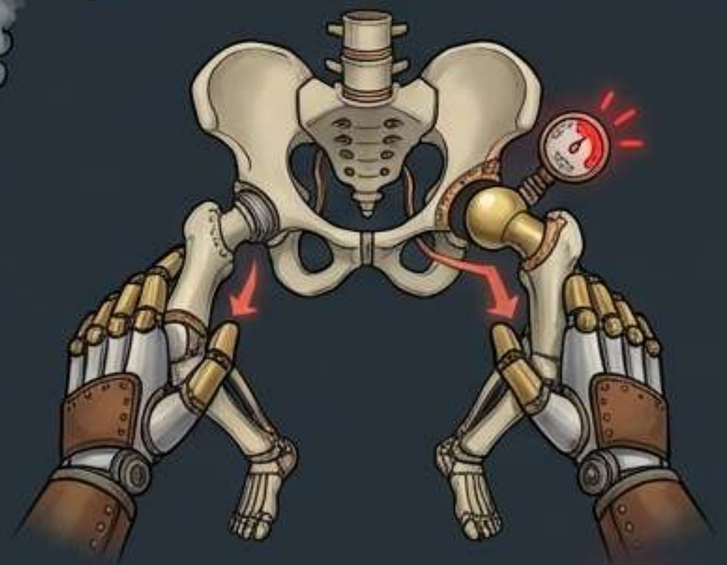


Palpable "Clunk" or "Click" = Positive for Dysplasia.

POSITIVE



"Barlow's Maneuver"



Dislocation on pressure.

DISLOCATED

DIAGNOSIS: CONGENITAL HIP DYSPLASIA.

Refer to Orthopedic Mechanist.

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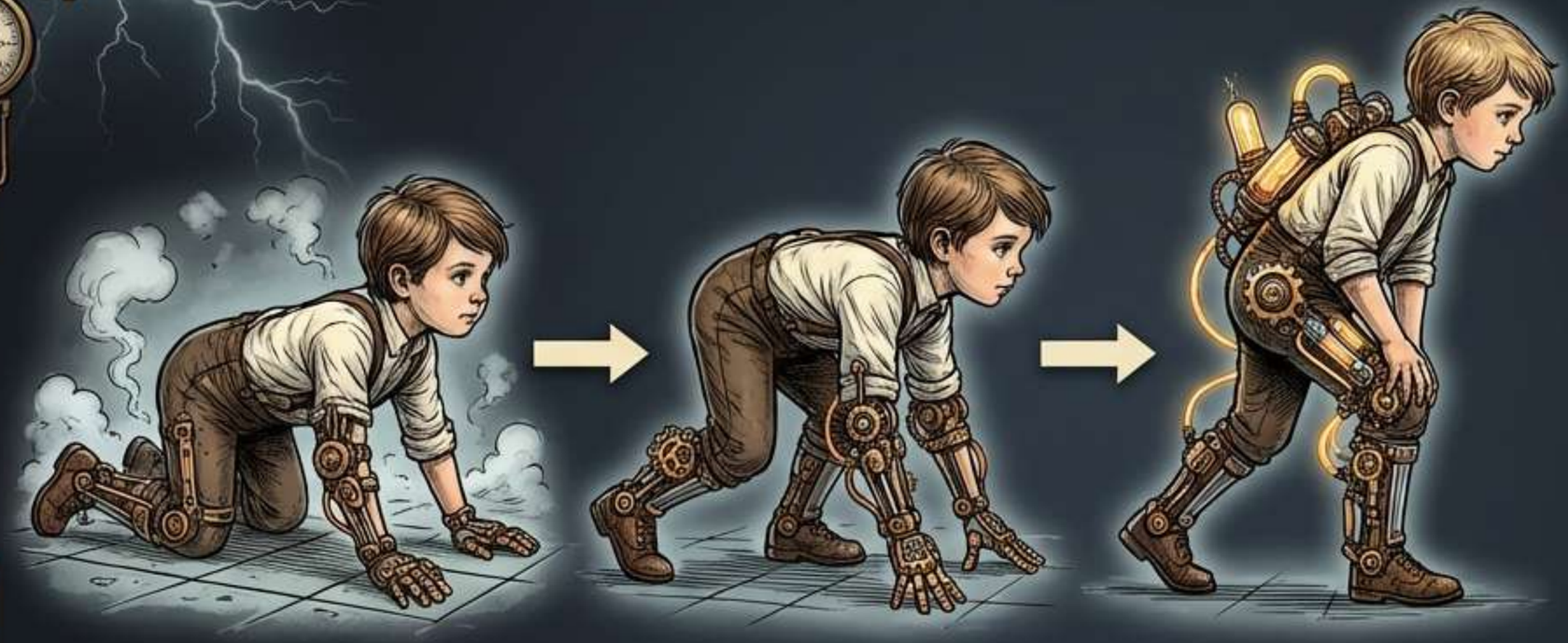
THE WALKING GEAR: MOTOR FAILURE



GOWERS' SIGN

Indicates Duchenne Muscular Dystrophy (Proximal Muscle Weakness).

TRENDELENBURG SIGN

Hip drops when standing on opposite foot. Gluteus medius weakness.

PRIORITY: FALL RISK / SAFETY.

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The Master Mechanist's Algorithm

Synthesized from Klimek Blue Book Logic.
Treat the patient, not the machine.

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RAPID REFERENCE CARD

MENINGITIS:	Brudzinski / Kernig	-> ISOLATE.
TETANY (Low Ca):	Chvostek / Trousseau	-> SEIZURE PRECAUTIONS.
APPENDICITIS:	McBurney / Rovsing	-> NPO.
PANCREATITIS:	Cullen / Turner	-> SHOCK ASSESS.
GRAVES:	Exophthalmos / Graefe	-> THYROID STORM.
DKA:	Kussmaul	-> SEVERE ACIDOSIS.

<u>Calibration</u>
Calcium: 8.4 - 10.2
pH: 7.35 - 7.45
Platelets: 150k - 450k

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THE
BLUE
BOOK
PROTOCOL COMPLETE
You have the diagnostics. You have the codes.
You know the
difference between a loose screw and a critical failure.
GO FORTH. FIX THE MACHINES. SAVE THE PATIENTS.
Prepared for NCLEX-RN Success.
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